



1. PATIENT INFORMATION

Practice Site	Date of Service	MRN
Patient Name	DOB	Payer
Evaluating Psychiatric NP	Collaborating / Supervising Physician	Dual-Eligible? ☐ Yes ☐ No

2. REASON FOR EVALUATION

☐ New Patient Intake	☐ Annual Re-Evaluation	☐ Post-Hospitalization / Post-ED
☐ Care Team Referral	☐ Self- / Family-Referred	☐ Other

If Other, specify: _____

3. PSYCHIATRIC HISTORY

Past Psychiatric Diagnoses
Current Psychiatric Medications
Past Psychiatric Hospitalizations / ED Visits
Substance Use History
Family Psychiatric History

This evaluation determines clinical eligibility for CMS care-management programs (CCM/PCM/APCM/RPM/RTM/TCM/BHI/CoCM) — see Page 3.



4. MENTAL STATUS EXAM

Appearance / Behavior

Speech

Mood

Affect

Thought Process

Thought Content

Cognition / Orientation

Insight / Judgment

fair/good/poor

5. STANDARDIZED SCREENING TOOLS

PHO-9

GAD-7

Cognitive Screen (MoCA/Mini-CoA)

AUDIT-C

6. SAFETY ASSESSMENT

Suicide Risk Level (C-SSRS or equivalent)

☐ None

☐ Low

☐ Moderate

☐ High — Same-Day Escalation Required

☐ Safety plan created

☐ Means restriction counseling provided

☐ Homicidal ideation screened — negative

☐ Emergency contact / crisis line reviewed

If HIGH risk: do not leave patient unattended. Initiate same-day psychiatric escalation / crisis protocol before proceeding.
988 Suicide & Crisis Lifeline — call or text 988. Document escalation actions in the clinical note.

**7. DIAGNOSES**

Primary Psychiatric Diagnosis (ICD-10)

Secondary / Comorbid Psychiatric Dx

Relevant Medical Comorbidities (ICD-10)

*e.g., diabetes E11.x, HF I50.x, COPD J44.x, CKD N18.x — drives CCM/PCM/APCM eligibility (CMS-HCC V28)***8. RISK STRATIFICATION SUMMARY**☐ D-SNP / C-SNP / I-SNP (dual or SNP status)☐ ≥ 2 chronic conditions☐ ED visit or hospitalization ≤ 12 months☐ Frailty / functional decline☐ High utilizer☐ Behavioral health diagnosis present**9. CARE MANAGEMENT PROGRAM DETERMINATION***NP clinical determination — mark Indicated + Consented for each program that applies. APCM is mutually exclusive with CCM / PCM / TCM in the same month.*

PROGRAM	INDICATED	CONSENTED	NOTES
Chronic Care Management (CCM) 99490/99439	☐	☐	
Complex CCM 99487/99489	☐	☐	
Principal Care Management (PCM) 99424–99427	☐	☐	
Advanced Primary Care Mgmt (APCM) G0556/G0557/G0558	☐	☐	
Remote Patient Monitoring (RPM) 99453–99458, 99445, 99470	☐	☐	
Remote Therapeutic Monitoring (RTM) 98975–99985	☐	☐	
Transitional Care Management (TCM) 99495/99496	☐	☐	
BHI — General 99484	☐	☐	
Collaborative Care Model (CoCM) 99492–99494 / G2214	☐	☐	



10. TREATMENT PLAN & RECOMMENDATIONS

Medication Changes / Orders

Care Coordination Needs

Psychiatric Follow-Up Interval

Next Review Date

11. CARE TEAM ROUTING

☐ Care Manager

☐ Pharmacist

☐ Social Worker

☐ PCP

☐ Behavioral Health Manager

☐ Billing / Coding

REMINDER: Time cannot overlap between CCM, RPM, BHI, or CoCM. APCM excludes concurrent CCM / PCM / TCM billing in the same month.
Each enrolled service requires independent documentation, clinical purpose, and time tracking.

Psychiatric NP Signature & Credentials

Collaborating Physician Signature (if required)

Date

Date